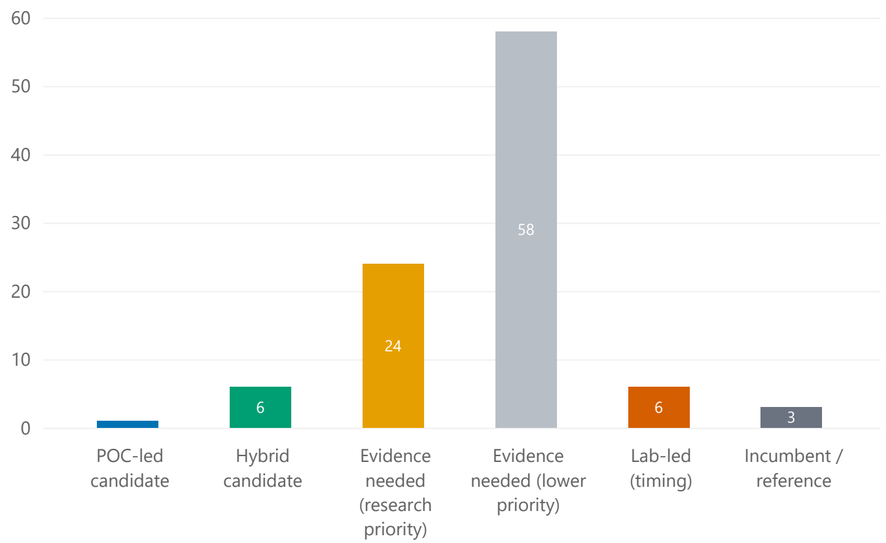
How much volume and revenue is at stake

Annual figures at Central volumes unless a scenario is chosen. Revenue is addressable if adopted, not a forecast.

Volume scenario (changes test volumes only)

- (●) Central
- ( ) Higher
- ( ) Limited

Services by category (all 98)



Addressable revenue if adopted, not a forecast.
Public sources only. Independent analysis.

Services **98**	Greenfield Services **95**	Revenue Category-Adjusted **£694,101**
Revenue Flat 25pc **£1,207,071**	Revenue Change vs Flat **-£512,970**	Sites With Timing Evidence **7**
Evidence Coverage **7.4%**	Candidate Sites **7**	Tests (selected scenario) **39,562**

Independent analysis from public sources. Revenue figures are addressable if adopted, not a forecast. Not affiliated with or endorsed by Genedrive plc.

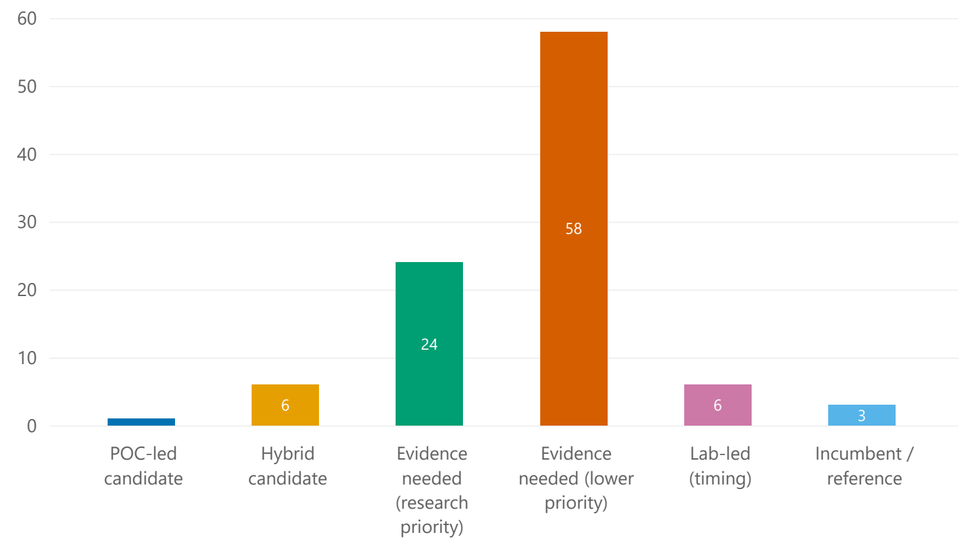
Where to start: ranking of 94 stroke services

- (●) Balanced
- ( ) Fit-led
- ( ) Volume-led

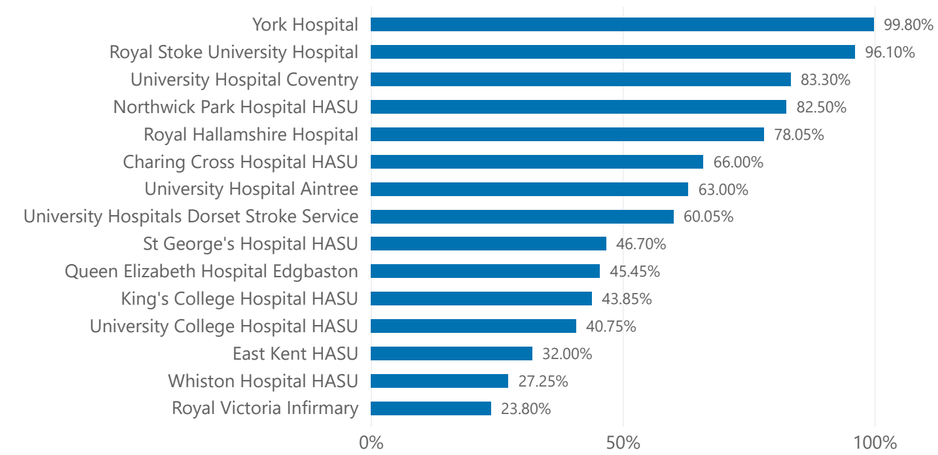
Site ranking, balanced lens (demand, time-sensitivity, POC suitability and readiness)

service_name	category	Rank (selected lens)
Northwick Park Hospital HASU	POC-led candidate	1
York Hospital	Evidence needed (research priority)	2
Royal Stoke University Hospital	Hybrid candidate	3
University Hospital Coventry	Evidence needed (research priority)	4
Royal Hallamshire Hospital	Evidence needed (research priority)	5
Charing Cross Hospital HASU	Evidence needed (research priority)	6
University Hospitals Dorset Stroke Service	Evidence needed (research priority)	7
University Hospital Aintree	Evidence needed (research priority)	8
King's College Hospital HASU	Hybrid candidate	9
East Kent HASU	Evidence needed (research priority)	10
University College Hospital HASU	Evidence needed (research priority)	11
Queen Elizabeth Hospital Edgbaston	Evidence needed (research priority)	12
St George's Hospital HASU	Hybrid candidate	13
Southampton General Hospital	Evidence needed (research priority)	14
Royal Victoria Infirmary	Evidence needed (research priority)	15
New Cross Hospital	Evidence needed (research priority)	16
Eastbourne District General Hospital	Evidence needed (research priority)	17
Whiston Hospital HASU	Evidence needed (research priority)	18
University Hospital of North Durham	Hybrid candidate	19
Cheltenham & Gloucester Hospitals	Evidence needed (research priority)	20
Leicester Royal Infirmary	Evidence needed (research priority)	21
Hull Royal Infirmary	Evidence needed (research priority)	22
Stepping Hill Hospital	Evidence needed (research priority)	23
Lincoln County Hospital	Evidence needed (research priority)	24
Northumbria Specialist Emergency Care Hospital HASU	Evidence needed (research priority)	25
Royal Devon and Exeter Hospital	Hybrid candidate	26
Royal Hampshire County Hospital	Evidence needed (lower priority)	27
Cumberland Infirmary	Evidence needed (research priority)	28
Ipswich Hospital	Evidence needed (research priority)	29
Royal Cornwall Hospital	Evidence needed (lower priority)	30

Services by category (all 98)



Share of 2,000 random weightings in which each site ranks top 10



Independent analysis from public sources. Revenue figures are addressable if adopted, not a forecast. Not affiliated with or endorsed by Genedrive plc.

Shortlist: why these seven sites

Seven sites reach POC-led or hybrid status because of their evidence. Rank reflects the numbers. They don't always agree (see Doncaster)

Service	category	Balanced rank	Volume rank	Fit rank	Top 10 rate	Tests/yr
University Hospital of North Durham	Hybrid candidate	19	23	12	14.1%	511
St George's Hospital HASU	Hybrid candidate	13	19	4	46.7%	489
Royal Stoke University Hospital	Hybrid candidate	3	7	2	96.1%	518
Royal Devon and Exeter Hospital	Hybrid candidate	26	38	21	7.7%	391
Northwick Park Hospital HASU	POC-led candidate	1	2	1	82.5%	571
King's College Hospital HASU	Hybrid candidate	9	16	8	43.9%	505
Doncaster Royal Infirmary	Hybrid candidate	64	65	62	1.8%	334

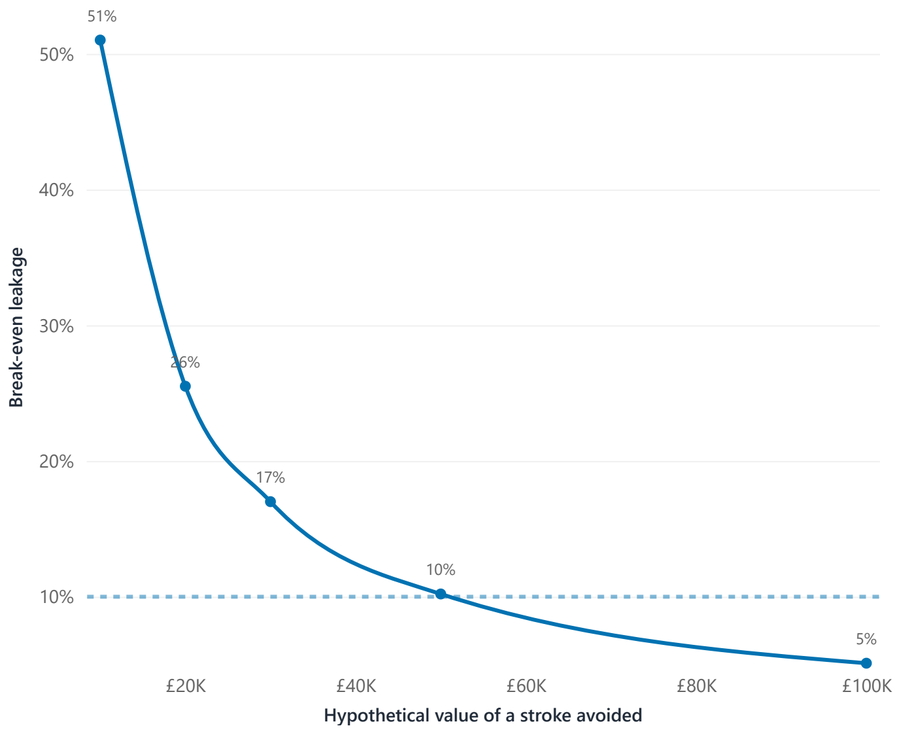
Balanced rank	Service	Evidence basis	Main caveat	Next step
1	Northwick Park Hospital HASU	Stage 2 review: timing evidence Strong, readiness High. No Genedrive implementation or rapid competitor found. Only site with Strong timing and 300+ tests a year, so POC-led.	The Strong rating comes from the Stage 2 review. The Oct 2026 evidence pass found no site-specific confirmation of route or turnaround (EV08).	Confirm whether stroke/TIA orders CYP2C19 and by which route; sample-to-result turnaround; discharge timing; whether rapid POC has been considered.
3	Royal Stoke University Hospital	Stage 2 review: possible referral-access gap, readiness High. No Genedrive implementation or rapid competitor found. Stays top 10 in about 96% of random weightings.	Referral laboratory, cost and turnaround not confirmed. Timing evidence is Possible, not Strong.	Check UHNM Molecular Diagnostics for the CYP2C19 referral laboratory, cost and turnaround; whether stroke/TIA teams order it; whether POCT has been considered.
9	King's College Hospital HASU	Stage 2 review rated timing Strong; downgraded to Possible (override E02): Synnovis 5 working days (about 7 calendar days plus transport) against an 8.0-day median stay is borderline. Readiness High.	Borderline timing gap: the lab route may be adequate for some patients. Ranks 9th balanced, top 10 in 44% of weightings.	Confirm whether clinicians currently order the Synnovis assay; order volume; real sample-to-result turnaround; whether rapid testing has been evaluated.
13	St George's Hospital HASU	National NHS England pilot lab-arm site (Dec 2024-Apr 2025, EV01). Pooled lab arm: 88% of patients needing clopidogrel were discharged before the result. Timing Possible, readiness Medium (override). Time-sensitivity score 88 is the highest of the seven.	The 88% is pooled across three lab sites, company-reported, and the route may have changed since. Readiness Medium keeps balanced rank at 13th; 4th on the fit-led lens.	Confirm current testing route and turnaround at the trust and whether the lab arm continued after the pilot.
19	University Hospital of North Durham	Stage 2 review: possible referral-access gap, readiness Medium. No Genedrive implementation or rapid competitor found. Demand score 77.	Unclear whether the trust offers or refers CYP2C19 testing at all. Top 10 in only 14% of weightings.	Confirm whether CDDFT offers or refers CYP2C19 testing, the performing laboratory and turnaround, and whether the stroke/TIA team orders it.
26	Royal Devon and Exeter Hospital	National NHS England pilot lab-arm site (EV01). Timing Possible, readiness Medium (override). Time-sensitivity score 64.	Mid-sized demand (about 390 tests a year); pilot evidence is pooled and company-reported. Top 10 in under 8% of weightings.	Confirm current route and turnaround; whether testing continued after the pilot.
64	Doncaster Royal Infirmary	National NHS England pilot lab-arm site (EV01, EV07). Doncaster and Bassetlaw tested 130 stroke patients Dec 2024-Apr 2025; 20% had a reduced-response variant. Timing Possible, readiness Medium (override).	Candidate on evidence, not on numbers: lowest volume of the seven (about 330 tests a year) and short stay, so ranks 64th. The 20% reduced-response rate is lower than the 29-33% seen elsewhere.	Confirm route and turnaround; test the lower local loss-of-function rate against the 32.1% NICE figure.

Independent analysis from public sources. Revenue figures are addressable if adopted, not a forecast. Not affiliated with or endorsed by Genedrive plc.

Speed case: what would rapid testing need to achieve to pay for itself?

Illustrative. The value per stroke avoided is a hypothetical willingness to pay, not a NICE threshold.

Break-even leakage by value of a stroke avoided



Per stroke value is an **estimated** figure not reflecting a published **NICE** statistic. At £50,000 per stroke, POC breaks even if it recovers about 10% of the patients the lab route misses. At £100,000, about 5%.

Cost per extra stroke avoided (£), by share of leaked patients POC recovers

category	5%	10%	25%	50%
POC-led candidate	£98,198	£49,099	£19,640	£9,820
Hybrid candidate	£100,386	£50,193	£20,077	£10,039
Evidence needed (research priority)	£98,592	£49,296	£19,718	£9,859
Evidence needed (lower priority)	£104,559	£52,280	£20,912	£10,456
Lab-led (timing)	£104,110	£52,055	£20,822	£10,411

At 10% recovery, each extra stroke avoided costs about £49,000 to £52,000 of test premium in every category. The category matters little, because the cost is set by how many patients the lab route misses.

How these numbers are built

Break-even leakage = extra test cost of POC over lab ÷ (value per stroke avoided × strokes avoided if all loss-of-function patients switched)
At £50,000: £2,099,197 ÷ (£50,000 × 411.4) = **10.2%**

Cost per extra stroke avoided = extra test cost ÷ (leakage × strokes avoided if all patients switched)

Leakage = the share of loss-of-function patients who would be switched to a better drug if the result arrived before discharge, but are missed on the lab route.
Strokes avoided come from the NICE event tables, using the working switch mix (a placeholder). The values per stroke are hypothetical.

Extra test cost if every greenfield site used POC (per year)

£2,099,197

Strokes avoided over 5 years if every LOF patient switched

411.4

Break-even leakage at a hypothetical £50,000 per stroke

10.2%

Independent analysis from public sources. Revenue figures are addressable if adopted, not a forecast. Not affiliated with or endorsed by Genedrive plc.

Commissioners: where the demand and evidence sit by ICB and region

Addressable revenue if adopted, not a forecast. Shows where evidence work would change the answer most.

By ICB (36 integrated care boards)

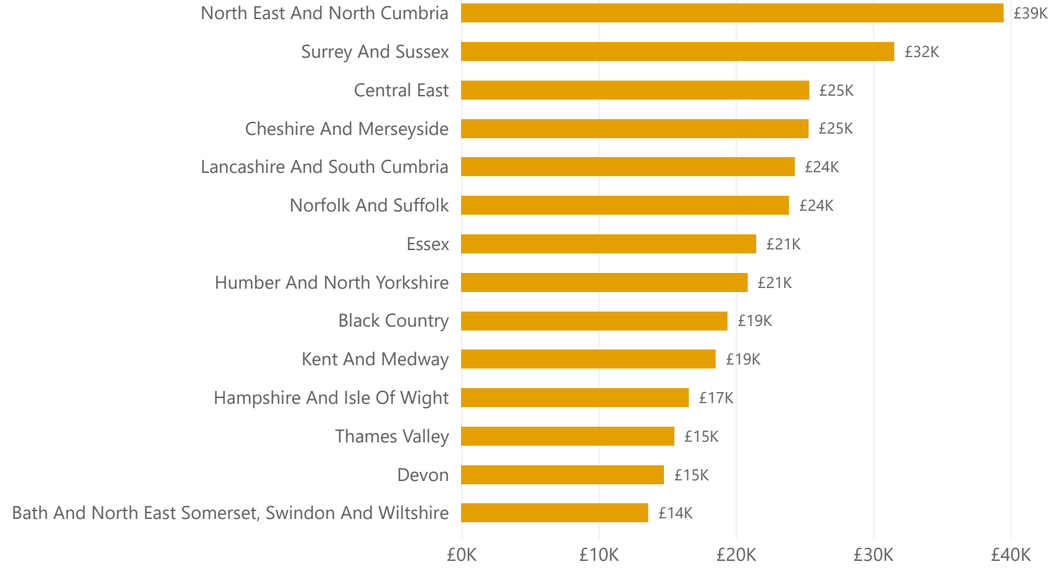
ICB region	services	Revenue (category-adjusted)	POC-led hybrid sites	Services with timing evidence
North East And North Cumbria	7	£58,698	1	1
West And North London	3	£47,892	1	1
Surrey And Sussex	8	£44,913	0	0
Cheshire And Merseyside	5	£31,919	0	0
Devon	4	£30,861	1	1
Central East	6	£30,798	0	0
Lancashire And South Cumbria	5	£30,091	0	0
Norfolk And Suffolk	5	£29,830	0	0
Essex	4	£25,447	0	0
Humber And North Yorkshire	3	£22,239	0	0
Black Country	3	£21,272	0	0
Kent And Medway	3	£20,707	0	0
South East London	2	£20,587	1	1
South Yorkshire	2	£19,905	1	1
Hampshire And Isle Of Wight	3	£19,393	0	0
Thames Valley	3	£18,682	0	0
Bath And North East Somerset, Swindon And Wiltshire	3	£17,421	0	0
North East London	2	£16,155	0	0
Staffordshire And Stoke-On-Trent	1	£15,865	1	1
Derby And Derbyshire	2	£15,227	0	0
Dorset	2	£15,181	0	0
South West London	1	£15,118	1	1
Nottingham And Nottinghamshire	2	£14,539	0	0
Greater Manchester	3	£14,229	0	1
Herefordshire And Worcestershire	2	£12,464	0	0
Total	**98**	**£694,101**	**7**	**9**

By genomic laboratory hub region (7)

Genomic lab hub region	services	Revenue (category-adjusted)	POC-led or hybrid sites	Services with timing evidence (all 98)
Central and South	20	£142,296	1	1
North East and Yorkshire	16	£117,840	2	2
East	16	£105,375	0	0
South East	14	£101,325	2	2
North Thames	9	£89,485	1	1
North West	13	£76,238	0	1
South West	10	£61,543	1	2
Total	**98**	**£694,101**	**7**	**9**

The hub runs the current lab **CYP2C19** test, so it is the incumbent route a rapid test would sit beside.

Top 10 ICBs by revenue that depends on evidence



Independent analysis from public sources. Revenue figures are addressable if adopted, not a forecast. Not affiliated with or endorsed by Genedrive plc.

Revenue at stake = flat 25% POC share minus the 10% default, for the 82 greenfield evidence-needed sites. Total £485,861.

Evidence and calibration: how far can the numbers be trusted?

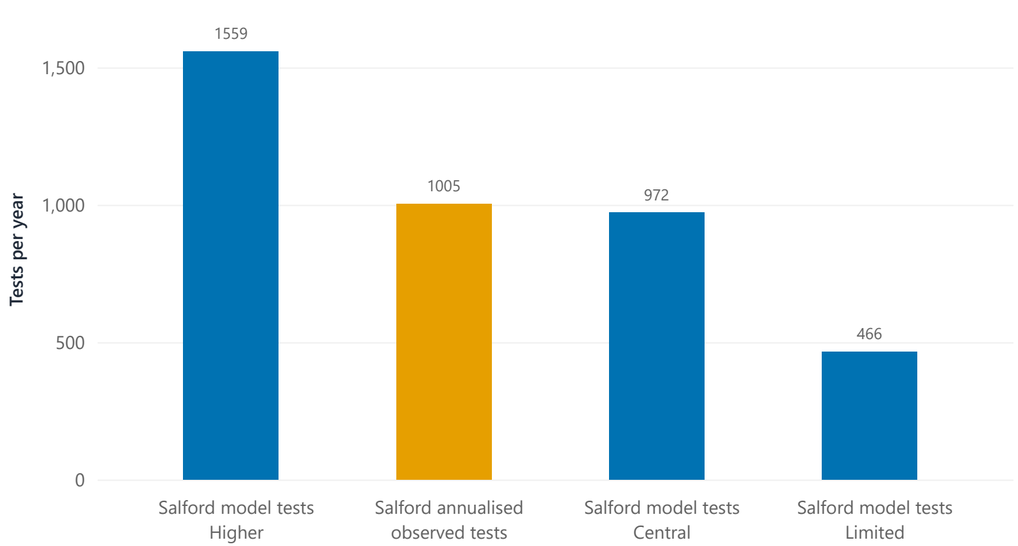
Published loss-of-function rates, a volume check against Salford, and the sites where new evidence would move the answer most.

Loss-of-function rate: published sources

Source	Rate	95% CI low	95% CI high
National pilot, POC arm (Salford)	33.0%		
Salford HASU (POC), first seven months	32.5%	28.71%	36.29%
National pilot, lab arm (3 sites)	29.0%		
NHS Tayside (lab), 2022-24	29.0%	27.15%	30.85%
Genomadix abstract, one UK centre	26.0%	16.20%	35.80%
Doncaster (DBTH) pilot	20.0%	13.12%	26.88%

NICE's model uses 32.1%. That is at the upper end of these sources (the lab-based ones sit near 29%). It changes strokes avoided and the break-even leakage, not Genedrive's kit revenue.

Salford: observed tests against model scenarios (tests a year)



Salford's observed volume is within about 3% of the Central estimate (1,005 against 972). This is one site, so it checks the scale of the model, not its accuracy across sites.

Evidence-needed sites with the most revenue at stake

service_name	ICB	category	Revenue at stake (£)
Eastbourne District General Hospital	Surrey And Sussex	Evidence needed (research priority)	£8,291
Leicester Royal Infirmary	Leicester, Leicestershire And Rutland	Evidence needed (research priority)	£9,391
New Cross Hospital	Black Country	Evidence needed (research priority)	£8,665
Northampton General Hospital	Northamptonshire	Evidence needed (lower priority)	£8,523
Queen Elizabeth Hospital Edgbaston	Birmingham And Solihull	Evidence needed (research priority)	£9,412
Queens Hospital Romford HASU	North East London	Evidence needed (research priority)	£8,703
Royal Derby Hospital	Derby And Derbyshire	Evidence needed (research priority)	£9,399
Royal Hallamshire Hospital	South Yorkshire	Evidence needed (research priority)	£8,851
Royal Victoria Infirmary	North East And North Cumbria	Evidence needed (research priority)	£10,044
Stepping Hill Hospital	Greater Manchester	Evidence needed (research priority)	£9,500
University College Hospital HASU	West And North London	Evidence needed (research priority)	£8,124
University Hospital Aintree	Cheshire And Merseyside	Evidence needed (research priority)	£9,099
University Hospital Coventry	Coventry And Warwickshire	Evidence needed (research priority)	£8,182
University Hospitals Dorset Stroke Service	Dorset	Evidence needed (research priority)	£10,340
York Hospital	Humber And North Yorkshire	Evidence needed (research priority)	£9,018

How to read this page

Volume: the model's Central estimate matches Salford's observed testing to within about 3%.

Loss-of-function rate: published sources range from 20% to 33%. We use NICE's 32.1%.

Still unverified: testing route and turnaround at most sites, and local rates. These are the first questions for any outreach.

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